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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 56 of 117



A 65-year-old woman status post heart transplantation presents for her tenth annual office visit with progressive shortness of breath. The patient denies any hospitalizations for acute decompensated heart failure and states adherence to a low salt, low fluid diet and her immunosuppression medications. Her routine annual testing is reviewed. 12-lead electrocardiogram reveals normal sinus rhythm at 75 beats per minute with a nonspecific intraventricular conduction delay of 120 milliseconds. Transthoracic echocardiogram shows a left ventricular end-diastolic dimension of 6.0 cm and a left ventricular ejection fraction of 45% with anterior and apical wall motion abnormalities and mild to moderate mitral regurgitation. She is referred for coronary angiography, which reveals 70% proximal stenosis in the left anterior descending artery, 80% stenosis in the mid left circumflex artery, and 70% stenosis in the proximal right coronary artery with diffuse and distal disease noted in all three vessels.

Current medications include atorvastatin 80 mg daily, tacrolimus 2 mg twice daily (recent trough level, 6.8; target level 5-8) and mycophenolate 500 mg twice daily. She has had no history of rejection episodes and was weaned off corticosteroids at the end of the first year after heart transplantation.

Which of the following is the next best step in management?

- A. Switch tacrolimus to everolimus
-  B. Refer for multivessel percutaneous coronary intervention
-  C. Begin evaluation for cardiac re transplantation
- D. Refer for coronary artery bypass grafting
- E. Switch atorvastatin to rosuvastatin

Testing Point: The Board-certified AHFTC specialist should recognize the indications for repeat heart transplantation.

Answer: C. Begin evaluation for cardiac re transplantation.

The patient has evidence of severe cardiac allograft vasculopathy (consistent with International Society for Heart and Lung Transplantation Grade 3 CAV since 2 or more primary vessels have > 70% stenosis). Cardiac re-transplantation is the only definitive management of advanced CAV; however, this approach may be limited by patient candidacy and scarcity of donor organs.

Answer A: While mTOR inhibitors have been shown to slow progression of early allograft vasculopathy, evidence for efficacy in severe vasculopathy is lacking. Switching from one mTOR inhibitor to another would not be predicted to have any effect.

Answer B: Percutaneous coronary intervention may be considered, especially when severe stenosis is seen with proximal primary vessels, but no improvement in long-term outcomes has been noted.

Answer D: Similarly, the use of surgical revascularization is limited by the diffuse and distal process of coronary arterial remodeling seen in CAV.

Answer E: There are insufficient clinical data to suggest that pharmacologic therapy can alter the natural history of CAV once it develops or has become this severe.

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